

DEE Roundtable 2026

From Insights to Impact

| Discussion Paper

Prepared for the DEE Roundtable 2026 – April 2026



About the Project

DEE Roundtable 2026: From Insights to Impact

This Roundtable builds on the impact of the inaugural Roundtable held in 2019, *Developmental and Epileptic Encephalopathies: Improving Care*, and the resultant publication in the European Journal of Paediatric Neurology, 'Fulfilling the needs of caregivers in delivering health services to children with developmental and epileptic encephalopathies' (Kelada et al. 2025).

This Project has involved the co-design and delivery of a national Roundtable that brings together key opinion leaders from across the Australian Developmental and Epileptic Encephalopathies (DEE) landscape – patient advocates, clinicians, researchers, policymakers, industry and others – to co-design practical, collaborative actions to improve outcomes for families affected by DEEs in Australia.

The objectives of the Roundtable are:

- 1 To identify and articulate the key challenges and priorities of DEE families across the care journey and life-course.
- 2 To define a set of clear, realistic, and meaningful goals for DEE and rare epilepsy advocacy across key domains.
- 3 To agree on a practical and coordinated pathway to deliver on these goals.

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- Kris Pierce – Project Chair and Co-Lead
Director, **SCN2A Australia** and Strategic Lead, **Rare Diseases NSW, UNSW**
- Associate Professor Katherine Howell
Neurologist and Neurology Research Lead, **Royal Children’s Hospital**, Clinician-Scientist Fellow, Neuroscience Group Leader and Epilepsy Team Leader, **Murdoch Children’s Research Institute**
- Kathryn Milne – Project Lead
Rare Insight Advisory

- Associate Professor Elizabeth Palmer
Clinical Geneticist, **Sydney Children’s Hospitals Network**, Associate Professor, **UNSW**
- Dr Neil Deacon
Founder, **Health Advance Connect**
- Meagan Cross
Co-Founder and CEO, **FAST Australia – Foundation for Angelman Syndrome Therapeutics**
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Acknowledgement of Country

The Roundtable Steering Committee acknowledges Aboriginal and Torres Strait Islander peoples as the First Australians. We recognise their cultures, histories and diversity and their deep connection to the lands, waters and seas.

About the Discussion Paper

This discussion paper aims to:

- Make the case that DEEs are a distinct condition category and, as such, warrant a distinct response in the areas of policy, research funding, clinical service design and disability support.
- Present examples of effective multi-stakeholder collaboration from Australia and internationally.
- Paint a picture of the Australian DEE landscape.
- Support attendee preparation for the DEE Roundtable 2026.



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1 | Executive Summary

Developmental and Epileptic Encephalopathies (DEEs) represent a group of severe, genetically driven neurodevelopmental disorders that manifest with epilepsy as one feature among a spectrum of complex, multi-system, life-long impairments.

In Australia, the DEE landscape is marked by significant advances in genetic diagnostics, research, and advocacy, yet remains challenged by fragmented care pathways, limited access to precision therapies, and insufficient national coordination.

This paper argues that DEEs are not merely a subset of epilepsy but are distinct neurodevelopmental conditions requiring a fundamentally different response in research, clinical care, and policy—a response that is not yet realised at a national level.

The current DEE ecosystem encompasses clinical care, diagnostics, disability and education systems, social services, research, policy, and a diverse organisational landscape. While Australia boasts world-leading research and advocacy groups, people living with DEEs and their families continue to face delayed diagnoses, inadequate access to multidisciplinary care, and significant psychosocial and financial burdens.

The disability and education systems, including the National Disability Insurance Scheme (NDIS) and school supports, often lack the flexibility and expertise to address the complex needs of this population. Access to emerging precision therapies is constrained by regulatory, reimbursement, and infrastructure barriers.

National and international examples suggest that uniting rare and ultra-rare diseases in distinct, broader disease categories can be a catalyst for action. Already in the Australian DEE ecosystem, we see pockets of excellence emerging as a result. This paper seeks to prepare Roundtable attendees to explore what could be achieved when all key DEE stakeholders work together to change the story for Australians living with DEEs and their families.

2 | Introduction

In recent years, Australia has seen important advances in DEE-related research, particularly in genomics, natural history studies, and early-phase drug development. (Scheffer et al. 2025)

Family-led advocacy, academic leadership, and growing industry interest have also helped to elevate DEEs on the national research agenda. However, despite this growing activity, translation into coordinated systems change has been limited. Families continue to experience long diagnostic odysseys, fragmented services, inconsistent care coordination, and limited trial readiness.

The current DEE ecosystem in Australia is highly fragmented. Researchers, clinicians, advocacy groups, and policymakers often operate independently, leading to duplication of effort, missed opportunities for collaboration, and a lack of shared strategic direction. While strong work is happening across many fronts, the absence of a cohesive national mechanism to connect and guide these efforts means progress remains uneven and fragile.

There are examples of the effectiveness of unification in Australia, most recently in the childhood dementia space, and internationally. Long-standing initiatives demonstrate the value of coordinated, cross-sector leadership, enabling knowledge sharing, policy engagement, collaborative research, and clinical trial mobilisation across multiple disorders under the DEE umbrella.

Australia can and should learn from these examples. While our context is unique, the need for a more structured, inclusive, and systems-focused approach is universal. The DEE Roundtable 2026: From Insights to Impact provides the platform to begin this work. It is an opportunity to bring stakeholders from across the DEE community together, break down silos, and set a collective agenda for national coordination and sustained impact.

What are Developmental and Epileptic Encephalopathies (DEEs)?

DEE are the most severe group of epilepsies. The International League Against Epilepsy (ILAE) formally recognised DEEs as a distinct category in its 2017 classification. This classification recognises “developmental consequences arising directly from the effect of the genetic mutation, in addition to the effect of the frequent epileptic activity on development.” (Scheffer et al. 2017)

Taking a person-centred perspective, individuals with DEE experience a wide range of *multi-morbidities*¹, in addition to their early-onset, usually drug-resistant, epilepsy. These multi-morbidities include intellectual disability, autism, psychiatric and behavioural consequences, movement and musculoskeletal disorders, feeding and gastrointestinal problems, sleep disturbances, and increased mortality, including an elevated risk of sudden unexpected death in epilepsy (SUDEP). DEEs are lifelong conditions, consistently associated with developmental slowing or regression. (Scheffer et al. 2024).

This classification is important, because it makes clear that DEEs impose a lifelong burden on affected individuals, families, and society. It also helps define what good care looks like for individuals with DEE and their families: managing a DEE well is not the same task as managing epilepsy well. The complexity and multi-system nature of DEEs necessitate lifelong, holistic and coordinated multidisciplinary management that extends beyond seizure control to address the full spectrum of developmental, medical, and psychosocial needs. (Scheffer et al. 2024)

¹ As per the Australian Bureau of Statistics definition, “Multimorbidity refers to living with 2 or more chronic conditions at the same time. People living with multimorbidity often have more complex health needs and experience poorer overall quality of life than people without multimorbidity. This makes preventing and managing multimorbidity an important focus for individuals, the health system and society as a whole.” See “Multimorbidity in Australia”, 15 July 2025, available at: www.aihw.gov.au/reports/chronic-disease/multimorbidity-in-australia/contents/about

Epidemiology and Burden in Australia

A recent epidemiological study estimated DEEs occur in 1 in 590 children below the age of 16 years. (Poke et al. 2023) Applied to Australia's birth cohort of approximately 300,000 per year, this equates to over 500 newly diagnosed children in Australia each year. Across the population, the point prevalence of DEE in children is 112 per 100,000 — a substantial and growing cohort requiring lifelong support.

More than 1000 genes have been implicated in DEEs, resulting in widely varying disease mechanisms (pathophysiology) and observable characteristics (phenotypes). More than 50% of cases have an identified genetic cause. (Scheffer et al. 2024)

DEEs are associated with high rates of morbidity and mortality, including increased risk of status epilepticus and SUDEP. (Donnan et al. 2023)

The burden on families and caregivers is severe and well-documented. More than 50% of carers report using mental health or family support services, and most describe chronic anxiety, financial strain and social isolation. Despite this, many needs remain unmet. (Kelada et al. 2025)

These are not just numbers. They represent real people living with DEE, and real families whose lives are turned upside down by these conditions.

They describe a population that it is currently invisible in mainstream policy settings. Yet, this population is significant enough, in size and disease burden, to warrant a coherent national response.



3 | The Central Provocation

The Australian health, disability, education and other systems typically fold DEEs into the broader category of epilepsy. The evidence does not support this, and neither does the lived experience of families.

DEEs are a group of severe neurodevelopmental conditions, due to a range of genetic or acquired disorders of brain structure or function. They present with epilepsy as one feature of a complex, multi-morbid disorder. The seizures are a downstream consequence of a genetic disruption to neurodevelopment, and in turn worsen development.

Treating the whole condition as an 'epilepsy problem' leads to care that is focused on only one aspect. Care is mainly via neurology clinics, primarily focused on seizure control, and poorly equipped to deal with the day-to-day challenges of living with a DEE.

The groundbreaking recognition of DEEs as a distinct category, led by Australia's Laureate Professor Ingrid Scheffer AO, is not a semantic exercise. This distinction has profound implications for how services are commissioned, how disability need is assessed, how research is funded, and how individuals and families experience the system.

The current response – fragmented, under-resourced, and lacking national coordination – falls short of meeting the complex needs of people living with DEEs.

A fundamentally different, person-centred, and system-wide response is urgently required.

Additional Supporting Evidence

Research Infrastructure Requirements Are Different

DEEs require natural history studies, biobanks, patient registries, outcome measure development, and development and trial of novel therapies. While some aspects of this work can be done across all DEEs, much of it (including natural history studies and novel therapies) needs to be individualised for each individual cause.

However, the rarity of each individual DEE, particularly for each individual genetic cause, means small patient numbers. Critically, given the need for novel therapies to improve outcomes, it is vital to note that standard epilepsy clinical trial infrastructure does not serve this need.

Randomised controlled trials with adequate statistical power are often not feasible for individual DEE genes, necessitating novel trial designs including n-of-1 studies and platform trials.

This is structurally closer to the ultra-orphan disease research model than to other types of neurological research. It demands coordinated national (and international) infrastructure that does not currently exist in a systematic way in Australia.

However, we have a key opportunity in that Australian researchers are global leaders in gene discovery and precision medicine in the DEE space. A number of Australian research institutes are leading efforts in disease modelling, natural history studies, and therapeutic development. Examples include the Epilepsy Research Centre at the University of Melbourne/ Austin Hospital, Florey Institute, the Murdoch Children's Research Institute (MCRI), and Telethon Kids Institute.

The Treatment Landscape Is Precision-Specific

The therapeutic pipeline for DEEs has shifted decisively toward gene-specific precision therapies that target the underlying cause. Antisense oligonucleotides (ASOs), gene replacement therapies, and targeted small molecules are being developed for individual genetic aetiologies.

"Unlike conventional antiseizure medications (ASMs) which only treat the symptoms (seizures) but have no effect on the underlying disease, precision therapies have the potential to suppress not only the seizures but also disabling comorbidities, including cognitive and behavioral abnormalities, which share the same causative mechanisms." (Wang et al. 2025)

However, by their nature, precision therapies are not necessarily transferrable or able to be repurposed, meaning a discovery in relation to one type of DEE does not necessarily translate to progress in relation to another.

Frameworks for moving novel therapies into clinical practice in Australia also have limitations that risk impacting access for Australian people with DEE to potentially beneficial therapies.

Australia's current Pharmaceutical Benefits Advisory Committee (PBAC) and Therapeutic Goods Administration (TGA) frameworks were not designed for ultra-rare populations with surrogate endpoints and limited placebo-controlled trial feasibility. DEE therapies are being assessed under frameworks built for common conditions. This is a major structural policy limitation. The Life Saving Drugs Program (LSDP) is currently the major pathway for access to funded, expensive, life-saving medicines. It currently provides access for approximately 400 patients nationally with rare disorders.

While their orphan drug designation threshold (fewer than 5 in 10,000) captures DEEs, other aspects of the assessment pathway for funding approval of novel therapies remains misaligned with the evidence base these conditions can realistically generate.

Australia's National Strategic Action Plan for Rare Diseases (2020) was a significant milestone — the first nationally coordinated effort for improving care broadly for people with rare disease, including processes for access to therapies in clinical practice. However, the 2023 Status Report by Rare Voices Australia found implementation uneven, with no equivalent to the UK's Highly Specialised Technologies programme or the EU's orphan medicinal product regulation providing a structured pathway for ultra-rare conditions.

It is imperative that approaches to Health Technology Assessment (HTA), regulatory approval, reimbursement and commissioning evolve rapidly as precision therapies approach clinical readiness.

Reflection

DEEs must be considered a distinct set of conditions, and not be subsumed within a broader epilepsy umbrella. They are severe and complex neurodevelopmental conditions that present with epilepsy as one feature of a multimorbid disorder. Most are genetically driven, each individually rare, but collectively common; management depends on the underlying cause.

The needs of people with DEE and their families are extremely high, and span health, disability, education and community sectors. Care is currently fragmented, and often lacking, resulting in enormous health, quality of life and economic impacts. The need to address this gap is an urgent one, for all people affected by DEE across the country. Additionally, there is a critical need for better treatments, which requires investment in research and emerging therapies.

4 | The Opportunity

Both closer to home and further afield, there are examples of organisations taking advantage of similar types of classifications of rare and ultra-rare disease, ones that unite them into a cohesive group.

In Australia, organisations like Mito Foundation, Rare Cancers Australia and Childhood Dementia Initiative show the potential for this type of national, and even global coordination.

These groups are advocating effectively for people living with various diseases, of diverse genetic origin, in the manner of a peak body, and examples abound of the achievements made possible through leveraging broader disease classifications as well as existing pockets of collaboration.

These three organisations are all registered with the ACNC, each as a HPC, and each with DGR status. They are each governed by a Board of Directors, and have established mechanisms for input by scientific and medical advisors, and lived experience advisors.

Types of Charities in Australia

The Australian Charities and Not-for-profits Commission (ACNC, www.acnc.gov.au) is our national regulator of charities. Registered charities can apply for charity tax concessions from the ATO, additional tax benefits (as a public benevolent institution (PBI), health promotion charity (HPC)), and for certain categories of deductible gift recipient (DGR) status. (Australian Charities and Not-for-profits Commission n.d.)

Below, we share some of the key characteristics and achievements of these organisations.

Characteristic	Mito Foundation	Rare Cancers Australia	Childhood Dementia Initiative
Patient Group	All types of mitochondrial disease, with more than 350 genetic causes identified to date	All rare and less common cancers	More than 140 rare genetic conditions that cause dementia in children
Focus	Drives awareness, research, diagnosis, and support to improve outcomes for people with mitochondrial disease.	Advances equity for rare cancer patients through advocacy, navigation, financial support, and national policy reform.	Unites 100+ disorders to drive recognition, research coordination, policy reform, and improved care for affected children.
Key Achievements	MSAC Application 1675 – Whole Genome Sequencing for the diagnosis of mitochondrial disease (Approved) Mitochondrial Donation Law Reform (Maeve’s Law) Bill 2021 (Passed)	\$4.36 million in funding over four years under the Government’s Australian Cancer Nursing and Navigation Program Australia’s first Optimal Care Pathway for rare and less common cancers MBS-funded molecular tests to diagnose rare or less common cancers	National Health and Medical Research Council Targeted Call for Research: Childhood Dementia 2026 (\$5m) Childhood Dementia Global Consensus Project

Reflection

In the case of each of these examples, there has been a clear pattern of progress:

Classification → Unification → Action

Growing recognition of the similarities between certain rare and ultra-rare diseases has enabled aggregation and unification of many conditions, in turn leveraging economies of scale to drive systemic change.

With the classification of DEEs as a distinct category in 2017, and with the rapidly growing number of conditions aggregating into this category – many more than represented by any of the three organisations described here – the DEE sector is ripe for adoption of this pattern of progress.

5 | International Approaches

International progress has been driven by entities that formalise DEEs (sometimes included in the term rare or complex epilepsies) as a policy and research category.

Initiatives such as the Rare Epilepsy Network (REN) in the United States, the European Reference Network for Rare and Complex Epilepsies (EpiCARE), and the European Association for Complex and Rare Epilepsies (CREA) demonstrate the value of coordinated, cross sector leadership in driving systemic improvements.

These platforms have enabled knowledge sharing, policy engagement, collaborative research, and clinical trial mobilisation across multiple disorders under the DEE umbrella.

Below, we share some of the key characteristics and achievements of these organisations.

Coordinating Body / Enabler	Rare Epilepsy Network (REN), USA	EpiCARE (European Reference Network for Rare and Complex Epilepsies)	European Association for Complex and Rare Epilepsies (CREA)
Approach	Cross disorder registry and research consortium.	European cross border clinical network.	Pan disease advocacy coalition.
Key Achievements	Built a first of its kind cross disorder registry; convened 150+ members and partners to accelerate natural history studies and trial readiness.	Coordinated specialist care across member states; harmonised referral pathways and enabled cross border clinical expertise and research collaboration.	United 40 organisations to amplify policy engagement, produce joint guidance, and raise visibility at EU policy fora.
Why DEE classification mattered	Aggregating many rare epilepsies created sample sizes and shared infrastructure that enabled multicentre research and cross syndrome trial platforms.	A pan DEE framing allowed small national cohorts to access specialist care and pooled expertise across countries.	Collective classification created political scale and a single voice for policy change and funding priorities.

Common enabling features across these models are:

- **A coordinating entity** (network, national programme, association) that holds responsibility for defining scope, standards, and priorities.
- **Shared definitions and data standards** that allow pooling of small cohorts into meaningful datasets for natural history studies and outcome development.
- **Cross disease research platforms and registries** that support novel trial designs (platform trials, n of 1, adaptive designs).
- **Policy mechanisms tailored to ultra rare conditions** (specialised HTA pathways, orphan incentives, cross border commissioning).
- **Integrated care pathways and specialist hubs** that provide multidisciplinary management beyond seizure control.

Reflection

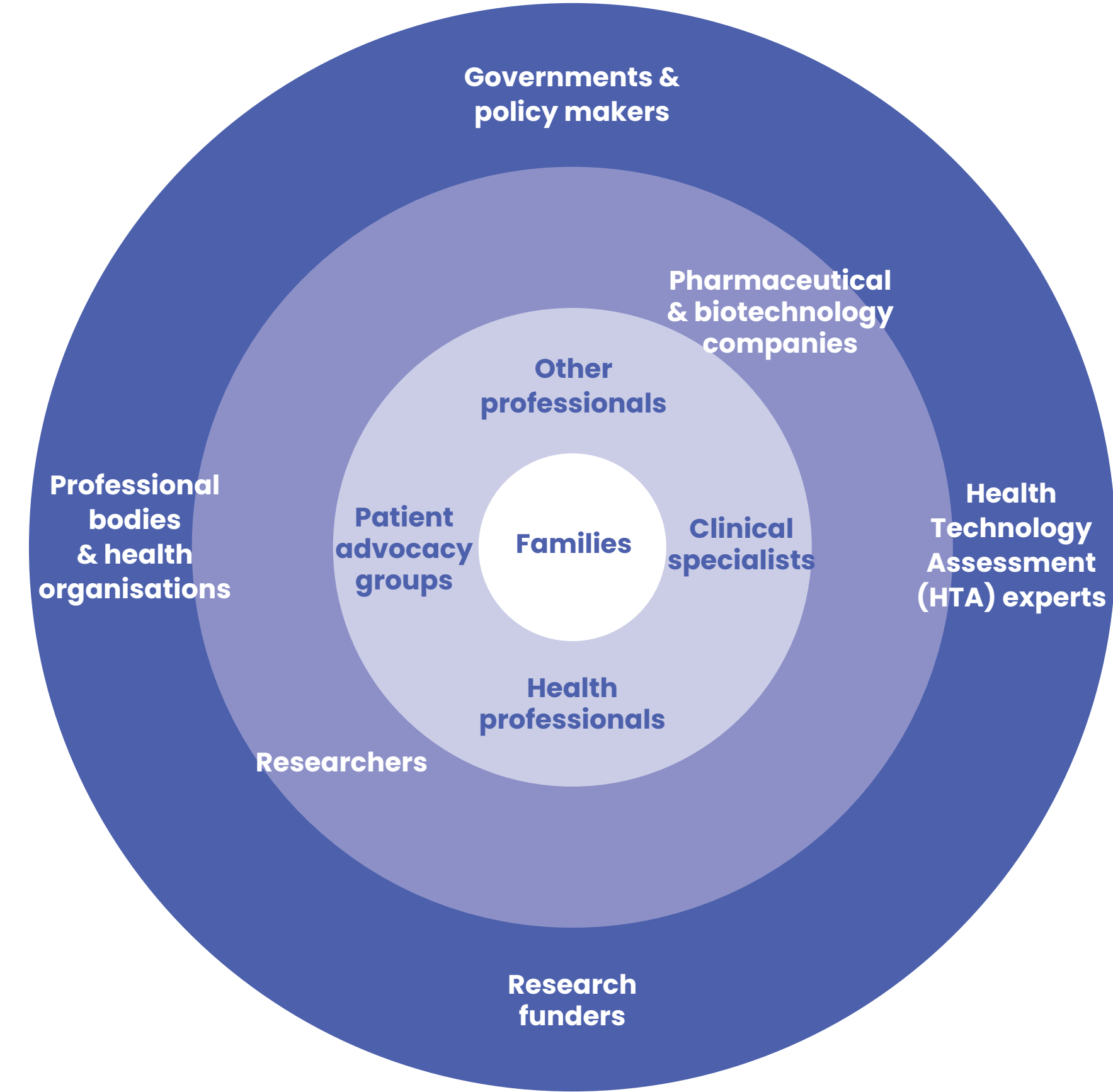
These international examples demonstrate the same pattern of progress described earlier: Classification → Unification → Action – indicating applicability in a DEE context.

They show that the presence of a formal coordinating body—one that defines DEEs as a distinct cohort, sets shared data standards, and links clinical care with research and policy—unlocks system level achievements.

There are many areas in the DEE space in which Australia is the global leader, and many examples of excellence across all domains. Some of these examples have been raised in this Paper. *Are you aware of pockets of excellence in the Australian landscape? What factors are enabling these? Are there barriers to their expansion?*



6 | The Australian DEE Landscape in 2026



Key stakeholder groups in the Australian DEE landscape include²:

Families: Individuals and families living with DEEs.

Patient advocacy groups: Groups representing and supporting these families, including DEE-specific groups.

Clinical specialists: With experience treating children and adults with DEEs, including paediatric or adult neurologists and epileptologists, clinical geneticists and neurodevelopmental psychologists.

Health professionals: With experience in providing medical and supportive care across the lifespan, including general practitioners, nurses, paramedics, paediatricians, palliative care specialists, and allied health professionals such as physiotherapists, occupational therapists, and speech pathologists.

Other professionals: The professionals with whom individuals and families living with DEEs interact with outside of the health system, including social workers, support workers, teachers and early childhood educators.

² Acknowledgement: This stakeholder list builds on and adapts work undertaken by Childhood Dementia Initiative, available at www.childhooddementia.org/globalagenda

Researchers: Neuroscientists and other researchers focused on fields relevant to DEEs, including basic science, clinical research, epidemiology, health economics, and implementation science.

Pharmaceutical and biotechnology companies: With interests in diagnostics, drug development, clinical trials, advanced therapy, cell and gene therapy, precision medicine, biomarkers or other innovative approaches relevant to DEEs.

Professional bodies and health organisations: With an interest in improving the landscape of care and research for individuals with DEEs, including medical colleges, nursing associations, allied health professional bodies, and healthcare quality improvement organisations.

Governments and policy makers: Including health ministers, department officials, and policy advisors working on rare diseases, epilepsies, children’s health, disability, and healthcare planning at local, national, and international levels.

Health Technology Assessment (HTA) experts: Including health economists, policy analysts, clinical specialists, and advisory bodies who evaluate the safety, effectiveness, cost-effectiveness, and broader societal impact of health technologies.

Research funders: Including government research agencies, private foundations, philanthropic organisations, industry partners, and international funding bodies committed to advancing DEE research and care.

Importantly, the Roundtable Steering Committee has sought representation from across these stakeholder groups in the DEE Roundtable 2026.

Reflection

Each of these stakeholder groups plays a critical role in the DEE landscape.

Which of these stakeholder groups is, could or should play a leading role in bringing these diverse groups together, driving coordination, resourcing efforts, and, ultimately, the fundamentally different, person-centred and system-wide response that is urgently required? Or, is there a place for the establishment of an alternative approach?

Patient Advocacy Groups

In the DEE landscape, individual gene-specific conditions are predominantly ultra-rare. This creates a structural challenge: each condition is too small to sustain its own infrastructure, yet the aggregate cohort is too large and too complex to be served by general epilepsy frameworks.

As the below table demonstrates, there are a number of DEE patient organisations operating in Australia, playing critical roles including supporting families, providing services, building infrastructure such as clinical registries, enabling research through fundraising and access to cohorts, and supporting industry with trial readiness.

All of the organisations in the table below are registered with the ACNC, most as either a public benevolent institution (PBI) or health promotion charity (HPC), allowing them to attract funding to achieve their charitable objectives.

Organisation	Focus
Angelman Syndrome Association Australia* angelmansyndrome.org	Supports, informs, educates, networks, promotes research, and advocates for families affected by Angelman syndrome.
Australian Foundation For CDKL5 Research afcr.org.au	Fundraises to support research, and supports families.
Foundation for Angelman Syndrome Therapeutics (FAST) Australia* cureangelman.org.au	Focused on development of a cure through support of global research, supporting families in accessing care through asNavigators, developing education and hosting conferences.
Rett Syndrome Association of Australia* rettaustralia.org.au	Supports parents, siblings and carers, collects and shares information, promotes awareness, facilitates and participates in conferences.
SCN2A Australia - scn2aaustralia.org	SCN2A Australia connects lived experience with research and policy. We partner with leading institutions including the Florey, MCRI and UNSW to support biobanking, natural history studies and therapeutic development, and work across the sector to strengthen consumer engagement, trial readiness and rare disease advocacy.
SLC6A1 Kids - slc6a1kids.org	Promotes the diagnosis, treatment and support of people affected by SLC6A1 neurodevelopmental disorder and other genetic epileptic encephalopathies.
STXBPI Australia - stxbp1australia.org	Raises money to accelerate research to find a cure for STXBPI disorders.
Syngap Research Fund Australia syngapaustralia.org	Raises awareness of Syngap, supports Syngap families and facilitates research via partnerships with key DEE research centres.
Tuberous Sclerosis Australia* - tsa.org.au	Provides people with Tuberous Sclerosis Complex with knowledge, support and connection. Develops educational resources and runs a telehealth nurse service.
WVOX Foundation (Australia) wvox.org/australia	Funds research, connects families with researchers and industry, raises awareness.

* These conditions are considered DEE-adjacent or conditional.

There are also informal patient groups in the Australian DEE landscape. For example, Genetic Epilepsy Team Australia (GETA) convenes an annual conference on Genetic Epilepsy for families, researchers and clinicians bringing together the latest research in genetic epilepsy and DEEs. However, it is not a registered organisation, does not have the ability to attract donations, and is not regulated under the ACNC framework. For some types of DEE, individuals have established a closed Facebook group that is not associated with an organisation, such as RNU4-2 / ReNU Syndrome Family Support Network (Australia).

Lived Experience Insight: *When asked what meaningful progress for the DEE community would look like, families shared visions of “an Australian voice”, of “one body that represents all of us”, and can connect families to one another. They spoke of “some sort of a national body, or a group” that can take on advocacy responsibility, as well as support system navigation.*

National and state-based organisations exist to support Australians living with and affected by epilepsy. As well as various state and territory-based organisations, there are also various collaborations and partnerships that are formalised to varying extents.

Lived Experience Insight: *Families felt that the offerings of these organisations are not necessarily suitable for DEE families due to their generalised focus.*

Consultation with these organisations confirmed that, while they may support DEE families with respect to the epilepsy-related elements of their lives, specifically, it is not within their scope to support them in relation to other morbidities associated with DEEs, or other challenges experienced as a result of living with such complex disorders.

Reflection

It is well-established that “[r]are disease organisations currently play a key role in raising disease awareness and providing critical person-centred information. These organisations are vital to the rare disease sector and often fill gaps in the system, not just in terms of awareness and education, but also care and support and, increasingly, in the research sphere.” (Australian Government Department of Health 2020)

We also know that rare disease organisations typically have narrow fundraising bases, compete with one another for funding – especially when multiple organisations exist to serve the same population group, and, as a result, rely almost exclusively on a volunteer workforce.

Of note, the business profiles of Mito Foundation, Rare Cancers Australia and Childhood Dementia Initiative buck this trend. Each organisation is staffed and their income trends upwards³. Further, the growing profiles of each organisation, and the repeat demonstrations of efficacy would suggest that their long-term sustainability is secured. Finally, consultations with these organisations for this Project revealed that, in stark contrast to organisations focused on a single rare or ultra-rare disease, that the vast majority of their fundraising income is from sources outside of their patient and carer community, offering significant opportunities for action.

³ This statement is based on analysis of Financial Reports lodged with the ACNC by each organisation, for Financial Years 2025, 2024 and 2023. Income increased for all organisations for all years, with the exception of a minor (3.22%) decrease in income between FY24 and FY25 for the Mito Foundation.

Conclusion

Developmental and Epileptic Encephalopathies are not a subset of epilepsy; they are genetically driven neurodevelopmental conditions that demand a fundamentally different response in research, clinical care, and policy.

Momentum is being gained in other rare and ultra-rare disease areas where a similarly broad classification has led to opportunities for unification across the landscape, and has in turn enabled action leading to systemic change. Furthermore, there are already examples of this pattern of progress in a DEE context internationally.

Australia stands at a crossroads: with world-leading research, advocacy, and clinical expertise, the opportunity exists to unify DEEs in build a nationally coordinated, person-centred, and equitable DEE ecosystem.

Achieving this vision will require authentic collaboration, investment in infrastructure and workforce, and unwavering commitment to the lived experience of people with DEEs and their families. The time for a new national response is now.



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